Activity 5: Whole-Person Wellness Programme Portfolio

Learning outcome: LO2 Objective: Design an inclusive portfolio for distinct employee needs Expected output: Whole-person programme architecture with component logic models and priorities Primary source: https://pmc.ncbi.nlm.nih.gov/articles/PMC8168167/

Goal

Convert a broad wellness ambition into a coherent prevention-to-recovery portfolio with access, ownership and evidence.

Real-world scenario

The published VHA Whole Health model emphasises whole-person goals and values, health coaching, wellbeing programmes and clinical care within a wider system. It illustrates how an ecosystem can orient support around what matters to the person rather than a fragmented menu of services.

HarbourView Hospitality, a fictional hotel group, wants one portfolio for 900 shift, office, food-service and facilities employees. Current offers are an annual health screen and ad-hoc fitness challenges with low shift-worker participation.

Evidence boundary

Public case facts and fictional learner data must never be mixed. Every numerical or organisational training input is labelled as an assumption.

Training assumptions / guardrails

- HarbourView, workforce data and current service performance are fictional.
- The organisation can use HPB resources where eligible but must validate current programme terms before commitment.
- Employee groups differ in schedule, physical demand, language, financial concerns and digital access.
- Health screening results will not be shared with line managers.

Questions

1. Which shared and targeted needs should the portfolio address?
2. How will prevention, early help, support and recovery connect?
3. What makes each pathway accessible and trustworthy for shift employees?
4. Which components should be piloted, scaled, partnered or stopped?

Detailed activity procedure

Step 1: Synthesize needs

Use the supplied personas and data to identify shared conditions and distinct barriers. Label evidence confidence and gaps.

Checkpoint: Record the evidence, calculation or decision produced in this step.

Step 2: Write portfolio principles

Agree voluntariness, privacy, inclusion, clinical boundaries, paid access, employee voice and job-design responsibilities.

Checkpoint: Record the evidence, calculation or decision produced in this step.

Step 3: Map the current offer

Place existing services across prevention, promotion, early help, treatment and recovery. Identify duplication and missing pathways.

Checkpoint: Record the evidence, calculation or decision produced in this step.

Step 4: Generate options

Design physical, mental, financial and social components plus work-design changes. Link every option to a stated need.

Checkpoint: Record the evidence, calculation or decision produced in this step.

Step 5: Build logic models

For each shortlisted component, map inputs, activity, reach, experience, condition or behaviour change and intended outcome.

Checkpoint: Record the evidence, calculation or decision produced in this step.

Step 6: Design access

Specify shift timing, languages, disability access, device alternatives, manager release time, confidentiality and navigation.

Checkpoint: Record the evidence, calculation or decision produced in this step.

Step 7: Prioritise the portfolio

Score need, equity, evidence, feasibility, risk, cost and measurability. Choose pilot, scale, partner or stop.

Checkpoint: Record the evidence, calculation or decision produced in this step.

Step 8: Create governance

Assign sponsor, programme owner, provider responsibilities, employee advisory input, safeguards and quarterly review.

Checkpoint: Record the evidence, calculation or decision produced in this step.

Deliverable

Whole-person programme architecture with component logic models and priorities

Acceptance criteria

Every component links to an evidenced need, the portfolio covers work conditions plus support, access is designed for distinct groups, clinical/privacy boundaries are explicit and each component has an owner and review measure.

Reflection and debrief questions

1. Which assumption changed your recommendation most, and why?
2. What evidence would you request before relying on the result?
3. Which stakeholder owns the next decision or mitigation?
4. Which review trigger or future exercise should follow?

Source note

Source accessed 16 August 2026: https://pmc.ncbi.nlm.nih.gov/articles/PMC8168167/